



PROCEDURE

Discharge Planning – For Inpatients

Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS MH Inpatient Areas

1. Aim

Outlines key aspects of discharge planning for inpatient mental health services.

2. Background

All North Metropolitan Health Service Mental Health (NMHS MH) clinicians actively plan as early as possible for the discharge of patients from inpatient care to facilitate effective and timely discharge as per the National Standards for Mental Health Services [Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation](#), and the legislative requirements of the [Mental Health Act \(MHA\) 2014](#). All NMHS MH Services are to develop local protocols, procedures, and tools such as checklists to inform and guide their staff to apply this policy.

Discharge planning should include input from:

- the patient;
- all members of the multi-disciplinary team (MDT);
- the patient's nominated person, carer, close family member or other personal support person and/or guardian/s, unless the patient expressly refuses their involvement, and is deemed to have capacity to make this decision;
- the service or private clinician who is designated to provide ongoing care and treatment for the consumer;
- update of micro alerts on the Psychiatric Services On-Line System (PSOLIS) and advice to referral facilities regarding any micro alerts.

Patients are to be involved in collaboration with the persons/agencies to whom information will be/has been provided, and the reasons for the referral e.g. to assist with home maintenance. If there are concerns about the release of information, this should be discussed with the patient and the nominated person, carer, close family member or other personal support person and/or guardian/s, and attempts made to resolve these concerns.

If the patient has been detained under the [Mental Health Act 2014](#) (MHA) the Clinical Head of Service or delegate is required to comply with notifications as detailed in the MHA 2014. The patient has the right to confidentiality and privacy, but should be encouraged to engage personal support persons in care planning. If the patient refuses consent to involve personal support persons, clinicians may still receive information but not release information without consent unless the information is specifically approved under the MHA 2014, the [Guardianship and Administration Act 1990](#), or the [Carers Recognition Act 2004](#) or there is significant risk of serious harm.

Note: If it is considered that there is significant risk to the patient or other person/s, then information may be provided to a nominated person, carer, close family member or other personal support person and/or guardian/s without explicit patient consent where the clinician's duty of care takes priority over the patient's wishes.

3. Risk

Discharge planning must occur in collaboration with the patient, nominated person, carer, close family member or other personal support person and/or guardian/s and the Service designated to provide ongoing care, to ensure effective transition between treatment settings (most commonly, inpatient and community) to reduce the risk of relapse and ensure continuity of care.

Discharge planning is a part of the continuum of care that commences at the time of admission and focuses on coordinating services effectively so that discharge occurs expediently and that the patient is not subjected to unnecessary delays.

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☒	Quality of Patient Care	☐
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☒	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Process

4.1 Predicted date of discharge

Every patient admitted to an inpatient unit within NMHS MH must have a predicted discharge date (PDD) set within 72 hours of admission, or upon transfer between wards.

The PDD is the date that the Medical Practitioner/Consultant Psychiatrist expects the patient to be ready for discharge, based on clinical judgement made by the medical staff, in collaboration with the MDT. This includes patients who, due to the nature of their condition, may have a prolonged expected length of stay.

The PDD is to be recorded on the mental health unit's (MHU's) Journey/Allocation Board, the patient's Care and Discharge Plans, Treatment Support and Discharge Plans (TSDP) and the electronic patient information systems (TOPAS and PSOLIS).

With consideration to patient confidentiality, identified nominated person, carer, close family member or other personal support person and/or guardian/s should be consulted within the discharge planning process and be kept informed of the PDD and time.

Discharge planning must be integral in the following stages of a patient's transition through an inpatient stay (for involuntary patients, see s. 186 of the MHA 2014):

- assessment;
- care and discharge plan development;

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- treatment/intervention and outcome measures;
 - review and follow-up.

The patient's discharge plan should address each of the following as a minimum:

- risk assessment;
- micro alerts, falls risk and aids or equipment if provided or on order, pressure injury risk or pressure injury management strategies or equipment provided or recommended if relevant;
- medication compliance;
- accommodation;
- relapse triggers and markers (e.g. Collaborative Action Plan);
- family and social supports, re-entry to work/ study if appropriate;
- follow-up within 7 days of discharge from the inpatient service by MH services;
- access to services that promote recovery and aim to minimise psychiatric disability and prevent relapse, based on current functioning of the patient;
- provision of a discharge summary to the patient or their carer/guardian if they lack capacity for medical decision making on the day of discharge;
- provision of a discharge summary to their GP on the day of discharge;
- re-entry plan for the patient about returning to hospital if needed usually outlined in the Collaborative Action Plan (Crisis Plan in PSOLIS) i.e. formal information of the process on how to return to the service, if and when required.

Patients should only be discharged when the Consultant Psychiatrist:

- is confident that the patient has achieved the objectives of the admission as articulated in their care plan;
- has reached an agreement within the team that the patient is at no increased risk of harm by being discharged.

4.2 Discharge summary

Every patient being discharged from NMHS MH Services should have a completed Discharge Summary in Notification and Clinical Summaries (NaCS) and uploaded onto PSOLIS and provided to the patient on their day of discharge. A copy must also be sent to the patient's General Practitioner (GP) and the referrer (if the referrer was not the patient's GP) and the service providing review and follow-up. A copy is retained in the medical record.

The treating team is to delegate a member of the team to discuss the written discharge information. Where appropriate and with consideration of the patient's confidentiality, the team is to ensure that the patient's nominated person, carer, close family member or other personal support person and/or guardian/s is provided with this information.

If it is considered that there is significant risk to the patient of not informing a carer, close family member or other personal support person, discharge information may be provided to the carer, close family member or other personal support person without explicit patient consent.

4.3 Discharge planning

To facilitate timely and accurate recording of the PDD, the following should commence within 72 hours:

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- a discharge plan and relevant sections of the electronic discharge summary;
 - risk screening for potential discharge delays and appropriate referrals made to mitigate this risk;
 - within 24 hours of admission, the ward clerk/or other person, must confirm with the patient that their GP details, and the patient's address and phone number are correct;
 - within 72 hours, key personal support persons must be identified and entered onto PSOLIS. Medical staff should also clarify who may be contacted and provided with relevant information, taking into account patient's wishes, best interest, legal status and decision-making capacity.

4.4 Discharge during weekends

If the patient is likely to be discharged during the weekend, the Consultant Psychiatrist and case manager responsible for overseeing the patient's care will ensure that instructions for discharge are available. Those instructions will include as a minimum standard:

- completed Collaborative Action Plan (CAPlan);
- completed Care Plan/Treatment Support and Discharge Plan (TSDP) or PSOLIS Management Plan;
- completed Discharge Summary recorded on PSOLIS and in the medical record;
- provisions documented for follow-up in the community within 7 days;
- management plan for the weekend recorded in the medical record;
- a list of conditions that must be met by the patient for discharge to occur;
- discharge medications available on the ward;
- re-entry plan (formal information provided to the patient of the process of returning to the service).

4.5 Unplanned discharge after-hours

If a patient seeks to discharge him or herself and a Medical Practitioner or Consultant Psychiatrist is not available for review, then the person in charge of the ward may complete a Form 2A – Order for Assessment of Voluntary inpatient, if indicated, under the MHA 2014 (see s.34), detaining the patient at the hospital for up to 6 hours from the time the person seeks to be discharged.

If the Duty Medical Officer (DMO)/Registrar decide that a patient may be discharged after-hours, the DMO/Registrar must contact the On-Call Consultant Psychiatrist and discuss the proposed discharge before it occurs.

A risk assessment and reason for discharge must be documented by the DMO/Registrar in the medical record.

The nominated person, carer, close family member or other personal support person and/or guardian/s must be notified before the discharge, unless there are specific exceptional clinical/practical/legal reasons not to communicate with the nominated person, carer, close family member or other personal support person and/or guardian/s about the discharge.

On discharge, the patient must be provided with a list of 'out of hours' contact numbers and other support services information, including that of emergency services (i.e., the Mental Health Emergency Response Line (MHERL)).

The reason for the discharge must be documented in the medical record.

It is the responsibility of the DMO/Registrar to ensure either that follow-up within 7 days is arranged or that this responsibility is delegated to the treating team. This must be documented in the medical record.

As for a planned discharge, a re-entry plan must be completed and made available in the medical record, on PSOLIS and a copy provided to the patient.

4.6 Follow-up within 7 days of discharge

Every MH patient discharged from inpatient care must be provided with telephone or face to face follow-up no later than seven (7) days (5 working days) after the date of discharge.

Designated hospital staff are to initiate contact with discharged patients within seven (7) days. A minimum of three (3) attempts to contact the discharged person are to be made, this may include, messages left on a mobile message bank with no call back from the patient.

In the event where contact with the patient has not been successful and confirmation of the details of follow up plans cannot be shared, clinicians are required to escalate to medical staff.

Medical staff will assess the risk and advise accordingly:

- no further action as low risk;
- contact Next of Kin (NOK);
- local community Assessment and Treatment Team (ATT) team to conduct a face to face one off review.

All of these interactions are to be recorded in PSOLIS. The Inpatient staff will also send out discharge notifications e-mails to our clinics to increase awareness of the discharge and requirements to follow up.

If a community mental health appointment is appropriate, it is the responsibility of the discharging inpatient service Consultant Psychiatrist to ensure that the referral has been received by the allocated community MH service/Duty Officer. It is best practice to refer early to an allocated community MH service. If a community appointment is not required, it is the responsibility of the discharging inpatient service to conduct a follow-up telephone call within 7 days (5 working days).

Overall, the inpatient service retains ultimate responsibility for the performance of the inpatient service, regardless of the task has been delegated to another area.

If the task has been allocated to the community MH service/Case Manager by the inpatient services, it is the responsibility of the allocated community MH service/Case Manager to ensure that referred patients are contacted within 7 days (5 working days) following their discharge date.

Patients being discharged from inpatient Mental Health Units (MHUs) must be given priority when outpatient/community MH clinic/ambulatory service appointments are being allocated.

A record of all patient contact should be entered onto PSOLIS, The contact should be recorded using the business rules applicable to local guidelines developed expressly for this purpose.

When the patient is discharged in absentia (e.g. not returned from leave), and is unable to be contacted (e.g. no known address and no phone) and their next of kin is unaware of their



whereabouts, risk is to be recorded on PSOLIS by the treating team member, as 'patient potentially at risk, risk not quantifiable due to NFA and no contact information'.

5. Compliance, monitoring and evaluation

The Clinical Director of each NMHS MH Program is to demonstrate compliance with the established relevant clinical indicators relating to discharge.

Measures to be met are:

- 100% of patients to have a PDD set within 72 hours of admission,
- 100% of patients to receive clinically appropriate contact within 7 days of the date of discharge from an inpatient MHU,
- <10% unplanned re-admissions within 28 days to an acute designated mental health inpatient unit,
- 90% of discharge summaries (including diagnosis) to be entered onto PSOLIS and a hard copy retained in the medical record on the day of discharge,
- 100% of discharge summaries (including diagnosis) to be entered onto PSOLIS and a hard copy retained in the medical record within 7 days of discharge,
- 100% of discharge/care planning to have involved patient and/or carer input as appropriate.

Related internal policies, procedures and guidelines

[NMHSMH Discharge Summary Completion – Inpatients Policy](#)

[NMHS Clinical Care of People who may be Suicidal Policy](#)

References

[NMHS Discharge Summary Policy](#)

[Admission, Readmission, Discharge and Transfer Policy \(MP 0058/17\)](#)

[Carers Recognition Act 2004](#)

[Guardianship and Administration Act 1990](#)

[Mental Health Act \(MHA\) 2014](#)

[Clinical Care of People Who May Be Suicidal Policy \(MP 0074/17\)](#)

[Principles and Best Practice for the Care of People Who May Be Suicidal](#)

[NMHS Clinical Care of People Who May be Suicidal Policy](#)

Policy Sponsor	Director of Clinical Services MHPHDS				
Policy Contact	Co-Director, Adult Inpatient MHS				
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NSQHS Standards Applicable:	 Std 1: Clinical Governance				
National Standards for Mental Health Services	Std 6: Consumers Std 7: Carers		Std 10: Delivery of Care 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, , Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
17/12/2014	V1	Director SQPU	Initial endorsement; developed in consultation with MHA2014 State Coordinator and Area SQRM, with acknowledgement to SMHS Discharge Planning Policy.
26/06/2018	V2.0	Senior Clinical Planning Officer, Corporate Services	Changes to discharge summary system and clinical coding frameworks, previous handbook that is cited is out of date. Coronial recommendation citing discharge summary to referrer and service providing follow-up, policy change to reflect this. Template updated, references and hyperlinks updated.
28/09/2022	V3.0	Co-Director, Adult Inpatient MHS	Scheduled review, minimal content change, update of template and hyperlinks
04/10/2022	V3.1	A/Policy and Audit Officer	Minor amendment – addition of NaCS in 4.2 Discharge Summary
09/02/2023	V3.2	Co-Director, Adult Inpatient MHS	Information updated regarding 7 day follow up from Inpatient Services

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